

ADR Identification Basics - Task 1

Case Report: Suspected Amoxicillin-Associated Adverse Drug Reaction

Case Summary

Female, 45 years. Presenting symptoms: rash, itching, nausea and dizziness. Medication history: amoxicillin (started 5 days ago) and paracetamol (as needed).

Timeline

Day	Event
Day -5	Amoxicillin started
Day -2	Symptoms began (3 days after starting)
Day 0	Symptoms persist (day 5 of therapy)

1. Is this likely an Adverse Drug Reaction?

Yes - a suspected ADR is likely. The symptoms began after amoxicillin was started and persist while treatment continues. Rash and itching are recognised reactions to penicillins; nausea is a common effect and dizziness can also occur. A report does not require proof of causality, only a reasonable suspicion.

2. Which drug is the suspect?

Amoxicillin is the primary suspect. Paracetamol is less likely because it was taken only as needed and the scenario gives no link between its doses and the symptoms.

3. Rationale

- **Timing:** Onset 3 days after the first dose is a plausible temporal relationship.
- **Known pattern:** Rash with itching suggests a possible hypersensitivity-type reaction to a penicillin antibiotic.
- **Unknowns:** No information on dechallenge, rechallenge, or alternative causes (infection, other medicines).
- **Causality:** Document as a suspected (possible/probable on the WHO-UMC scale) amoxicillin-associated ADR, not as definite causality.

4. What should happen next?

- Refer the patient to the prescriber promptly; the prescriber should decide whether to stop or withhold amoxicillin.
- Check for serious signs: facial or lip swelling, breathing difficulty, widespread blistering. Dizziness together with a rash needs attention.
- Record the reaction and the patient's penicillin allergy status.
- Report the case through the national pharmacovigilance system (PvPI in India).

Conclusion: Suspected amoxicillin-associated ADR; paracetamol unlikely; withhold amoxicillin pending prescriber review and report the case.